INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Relugolix: daily hormone treatment

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00830
Version 3

Loading dose, then once daily

Treatment stages

01 **Daily hormone treatment**
Loading dose, then once daily

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

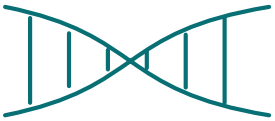
How this hormone treatment works

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

GNRH RECEPTOR ANTAGONIST

Relugolix

Orgovyx



Blocks the signal that makes testosterone

Relugolix blocks gonadotrophin-releasing hormone (GnRH) receptors in the pituitary gland. This lowers the signal to the testes and reduces testosterone without the initial surge seen with GnRH agonist injections. It does not normally need an anti-androgen solely to prevent a starting flare.

GnRH receptor blocked → Signal to testes reduced → Testosterone falls

What to watch for

If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Trials support the discussion but do not predict an individual outcome.

In the HERO trial, sustained testosterone suppression from Day 29 through Week 48 occurred in 96.7% receiving relugolix and 88.8% receiving leuprorelin. This was a hormone-suppression endpoint, not a comparison proving longer survival. The trial does not establish that this is the best treatment for every person.

CHAPTER 02

Your schedule, tests and appointments

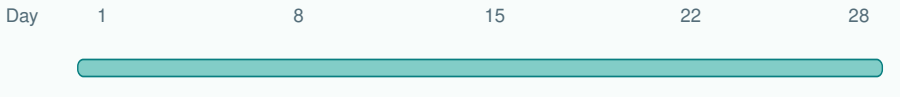
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Relugolix: daily hormone treatment

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

LOADING DOSE, THEN ONCE DAILY

Daily hormone treatment

Duration set by your treatment plan

Relugolix 360 mg on the first day, then 120 mg once daily without planned breaks. Your specialist sets the duration and any prescribed changes.

Androgen deprivation therapy (ADT) lowers testosterone, a hormone that can drive prostate cancer. This medicine is your ADT. Additional cancer tablets, chemotherapy or radiotherapy are separate decisions based on your cancer and overall health. Keep treatment going until your team changes the plan; do not stop because prostate-specific antigen (PSA) has fallen.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00830 v3: official regimen · Orgovyx: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

ECG and bone-density assessment

If part of your plan

WHEN

Baseline ECG is required for relugolix. Other ECGs and bone-density scans are based on risk and clinical need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Relugolix: daily hormone treatment: treatment and checks

Adults with advanced hormone-sensitive prostate cancer, following specialist assessment.

Relugolix 360 mg on the first day, then 120 mg once daily without planned breaks. Your specialist sets the duration and any prescribed changes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal. An ECG (heart tracing) is also a baseline NCCP test.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	Swallow tablets whole, with or without food, at about the same time daily. If no more than 12 hours late, take the missed dose when remembered; if more than 12 hours late, skip it and resume the next day. Never double the dose.	Keep an accurate medicine / injection record.	If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.	Continue until the team changes the plan.

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

1



Treatment and review

Loading dose, then once daily

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / **act:** Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal. An ECG (heart tracing) is also a baseline NCCP test.

STEP 2

Taking or receiving treatment

Swallow tablets whole, with or without food, at about the same time daily. If no more than 12 hours late, take the missed dose when remembered; if more than 12 hours late, skip it and resume the next day. Never double the dose.

STEP 3

Tests and reviews

Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

STEP 5

Leaving with a clear plan

Confirm the exact next dose or appointment, duration and verified daytime and out-of-hours contacts.

Your exact treatment schedule

Tests and preparation

PSA and testosterone

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily tablets	 **Days 1–28** Relugolix 360 mg on the first day, then 120 mg once daily without planned breaks. Your specialist sets the duration and any prescribed changes.

Prescribed schedule: Relugolix 360 mg on the first day, then 120 mg once daily without planned breaks. Your specialist sets the duration and any prescribed changes.

How this fits: Androgen deprivation therapy (ADT) lowers testosterone, a hormone that can drive prostate cancer. This medicine is your ADT. Additional cancer tablets, chemotherapy or radiotherapy are separate decisions based on your cancer and overall health. Keep treatment going until your team changes the plan; do not stop because prostate-specific antigen (PSA) has fallen.

Early safety: If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.

> **Why these bands stay broad:** The calendar shows a review window; tablets continue every day without an automatic rest week. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00830 v3: official regimen ↗

> **There are no guaranteed "safe days".** Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

Read the whole review window as text

Day 1: First day: loading dose

Take 360 mg only as prescribed when starting or when your team directs a reload. Confirm the dose and interactions.

Day 2–28: Daily maintenance tablets

Usual dose 120 mg daily, including weekends. Continue beyond this review window. Swallow tablets whole, with or without food, at about the same time daily. If no more than 12 hours late, take the missed dose when remembered; if more than 12 hours late, skip it and resume the next day. Never double the dose.

Support around the medicines

1 Sickness support

This treatment has minimal sickness risk. Routine preventive anti-sickness medicines are not usually required; agree a plan if symptoms develop.

2 Bone and muscle health

Discuss fracture risk, bone-density testing when indicated, calcium intake, vitamin D and suitable resistance and aerobic exercise. Supplements and bone medicines are individual decisions.

3 Sexual health, mood and hot flushes

Tell the team which effects matter most. Support is available for sexual function, sleep, mood and vasomotor symptoms.

4  Medicine review

Bring all prescriptions, over-the-counter medicines and supplements. Ask pharmacy before starting or stopping anything.

Why hormone treatment is used

Treatment goals and choices

Additional treatment

In metastatic hormone-sensitive cancer, many suitable patients benefit from an additional androgen-pathway medicine, and sometimes chemotherapy. Radiotherapy can have a different role in localised disease, selected metastatic disease or symptom relief. Your specialist explains the options for your setting.

Deciding together

Discuss expected benefit, uncertainty, side effects, alternatives including surgical castration where appropriate, and the option of declining treatment. No one duration fits every setting.

Questions worth asking

Do I need another cancer medicine or radiotherapy alongside this?

Ask how this hormone treatment fits the plan for your disease. Additional hormone-pathway medicines, chemotherapy and radiotherapy have different roles; they are not automatically needed in every setting.

How can we protect bone, heart, sexual and mental health?

Ask for a plan covering exercise, bone and fracture risk, blood pressure and glucose, sexual symptoms, mood and sleep. Tell the team about changes early so support can be offered.

Who can I contact urgently?

Use the verified daytime and out-of-hours numbers supplied by your treating hospital. For severe breathing difficulty, collapse, a seizure or severe chest pain, call 112 or 999. Do not use the example numbers in this draft.

> **Try explaining the plan back:** Explain your starting dose and usual schedule. Explain what to do after a missed dose or appointment. Name your next tests and your urgent contact route.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Loading dose, then once daily

Active medicines: Relugolix

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order. Agree symptom treatment if needed.

Additional phase-specific support

- Swallow tablets whole, with or without food, at about the same time daily. If no more than 12 hours late, take the missed dose when remembered; if more than 12 hours late, skip it and resume the next day. Never double the dose.
- Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and electrocardiogram (ECG) are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.
- If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.
- Bone health, calcium and vitamin D according to individual needs and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

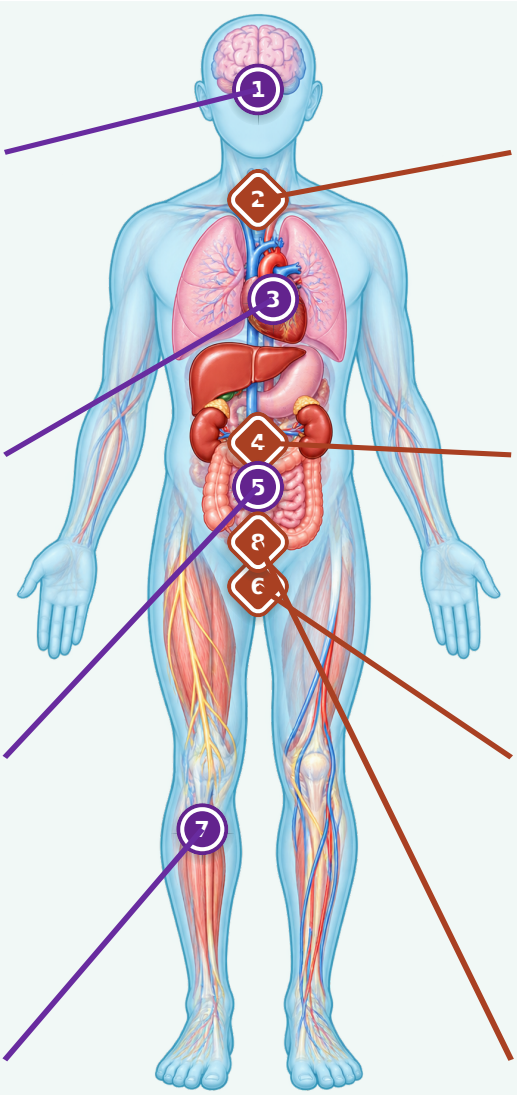
You do not need to know which medicine caused a symptom before calling.

1 Mood, sleep and energy
Relugolix
Report severe low mood, marked fatigue or sleep problems. Ask for support early.

2 Hot flushes and sweating
Relugolix
Hormone suppression can cause hot flushes. Ask about practical measures and treatments if troublesome.

3 Heart and blood pressure
Relugolix
Keep planned checks. Chest pain, fainting or severe breathlessness needs urgent assessment.

4 Weight and blood sugar
Relugolix
Blood glucose, weight and cardiovascular risk need review; suitable activity and diet can help.



5 Tablets and digestion
Relugolix
Report persistent diarrhoea, vomiting or difficulty taking tablets.

6 Sexual health and fertility
Relugolix
Reduced desire and erection difficulties are common. Discuss fertility and contraception; ADT is not reliable contraception.

7 Bones and muscles
Relugolix
Ask about bone assessment, resistance exercise, falls prevention, calcium intake and vitamin D needs.

8 Urine and early symptoms
Relugolix
Report difficulty passing urine or worsening pain. New leg weakness or loss of bladder/ bowel control is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP regimen-specific supportive care**
   https://healthservice.hse.ie/documents/6431/830_v3_Relugolix.pdf
2. **Orgovyx: product information**
   https://www.ema.europa.eu/en/documents/product-information/orgovyx-epar-product-information_en.pdf
3. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
4. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **NCI: hormone therapy for prostate cancer**
   https://www.cancer.gov/types/prostate/prostate-hormone-therapy-fact-sheet